Franchise Owner Authority Guide

Minnesota + North Dakota retail pharmacies | Federal controlled-substance and HIPAA baseline | **Current through 1 October 2025**

DECISION: Default owner ordering authority and patient-level access to OFF. Enable only after role approval, current documentation, and the applicable regulated authority are confirmed. This is a Goldfinch safeguard; it does not replace legal requirements.

The simple rule

LAW — must: A franchise, corporate title, check-signing authority, or funding responsibility does **not** itself make someone the pharmacy license/permit holder, DEA registrant, pharmacist-in-charge (PIC), pharmacist, authorized controlled-substance signer, workforce member, or authorized PHI user. Urgency, a vendor deadline, inventory pressure, financial ownership, or an owner instruction cannot cure missing authority.

Role	What it is — and is not
Franchise owner / entity officer	Business authority only. May sign a truthful owner/applicant or entity submission *if* that form calls for it and the signer is authorized for the entity; may not sign a PIC, pharmacist, DEA-registrant, or authorized-purchaser attestation merely because of ownership.
Licensed pharmacy / permit holder	The approved entity and **location** to which state licensure/permit attaches. A franchise relationship or another store's permit is not the local license.
DEA registrant	The registered pharmacy location for controlled substances. Each location must have the applicable registration; another store's registration is not portable.
PIC / pharmacist	PIC remains accountable for lawful practice, supervision, security, records, and operation. Delegation of a business task does not transfer professional judgment or regulatory accountability.
Authorized purchaser / signer	A person with the specific authority required for that transaction; title is not enough. Controlled-substance authority is checked against the registrant, POA, and personal credential where applicable.
Workforce member / business associate	A workforce member may use PHI only for an assigned permitted function and minimum necessary amount. A separate service provider is a business associate only under the required relationship; a BAA is not permission to browse.

Owner / PIC boundary

LAW — must: An owner may not direct the PIC or another pharmacist to violate law, override professional judgment, falsify records, or use an unauthorized signer. The PIC is not the owner's purchasing delegate. **GOLDFINCH SAFEGUARD:** document the assigned function, entity role, approver, scope, and end date before enabling any exception.

Controlled-substance ordering & receipt

Never: sign another person's name; use or lend another person's CSOS certificate/credential; treat a copied signature as authorization; or recreate an order under an unavailable person's identity. These are stop conditions.

Transaction	Legal boundary — must	Goldfinch operating control
Schedule I–II order	DEA Form 222 applies to Schedule I or II orders. In retail practice, Schedule I is ordinarily not ordered; Schedule II ordering still requires the registrant or a person holding the registrant's valid DEA power of attorney (POA) **before** signing. An ordinary purchase-order signature is insufficient.	Keep a current signer/POA roster by DEA-registered location; independently verify before activation and revalidate periodically.
CSOS order	CSOS is the electronic controlled-substance ordering method. It does not bypass registration-linked authority or Form 222 concepts. The certificate is individual—not a shared store credential—and must be current for the authorized signer.	Disable access promptly when employment, role, authorization, registration, certificate, or relevant status ends or changes.
Schedule III–V order	Form 222/CSOS is not generally the ordering instrument for Schedule III–V. They remain controlled substances: use the properly registered pharmacy's security, records, and authorized-purchasing controls.	Use assigned purchasing authority; do not treat the owner's ordinary grocery purchasing authority as a drug-inventory authorization.
Noncontrolled business goods	Ordinary business purchasing may be signed under entity authority. Prescription-drug purchasing still must be for the licensed entity and within assigned authority; it is not professional authority.	Separate general procurement permissions from pharmacy inventory permissions.
Receipt / discrepancy	The pharmacy must maintain required controlled-substance receipt, reconciliation, security, and record controls. Owner involvement never replaces them.	Match shipment to authorized order records; secure product and escalate a discrepancy to PIC and controlled-substance compliance.

If authority is unavailable

LAW — must: stop the regulated action until current authority is confirmed. Do not "borrow" a signature, credential, registration, or other store's DEA authority. **Escalate:** PIC/pharmacy operations for immediate practice and inventory controls; controlled-substance compliance for Form 222, CSOS, POA, registration, or alternate-supply questions. A currently authorized person may act only within current scope. A lawful transfer, alternate supplier process, or delay may be considered only when permitted and approved—not promised in every circumstance.

POA lifecycle

LAW — must: controlled-substance POA is a documented grant from the DEA registrant, retained with required records, revocable, and limited to its scope. Check it; do not infer it from job title. **GOLDFINCH SAFEGUARD:** retain a status check showing grant, scope, location, revocation/termination review, and reviewer.

State structure, ownership & professional accountability

Minnesota	North Dakota
LAW — must: Nonpharmacist ownership can be part of a lawful structure, but it does not confer professional, DEA, or PHI authority. The pharmacy must remain licensed, have an appropriately licensed PIC, and meet applicable ownership/applicant disclosure and operational requirements. Confirm state action before a location, owner/entity, control, or PIC change; it may require notice, approval, or a new application.	**LAW — must:** Pharmacist-ownership restrictions apply. A new nonpharmacist grocery owner cannot assume a franchise agreement authorizes pharmacy ownership or operation. Any statutory exception, grandfathered arrangement, or board-approved structure is entity-specific and nontransferable without verification. Before any ownership/control/entity/PIC change, obtain Board/legal review of the approved structure and required filing/approval.

TIME CHECK — 1 Jan 2024 → 1 Oct 2025: The reviewed core rules on DEA location/signer authority, Form 222/CSOS/POA, HIPAA role-based minimum-necessary access, PIC accountability, Minnesota nonpharmacist ownership posture, and North Dakota pharmacist-ownership restrictions remained materially stable for this guide. The post-signature-incident default-off workflow, independent verification, revalidation, and exception record are Goldfinch policy clarifications, not claimed changes in law.

Patient information: visibility without browsing

LAW — must: Ownership does not create a right to view patient names, prescriptions, profiles, claims, counseling information, or other PHI. Internal access requires covered-entity workforce status, an assigned permissible pharmacy/health-care function, authorization, and the minimum amount needed. Access must be controlled and auditable; suspected unauthorized access is reported promptly. Disclosure to an outside owner, adviser, affiliate, or service provider needs an appropriate legal basis. A business-associate relationship is service-limited, not blanket browsing permission.

Not sufficient by itself: curiosity; employee, family, or personal relationship; a dispute; marketing; general owner oversight; or confidentiality promise. **GOLDFINCH SAFEGUARD:** meet ordinary owner oversight with aggregate, de-identified, limited, or otherwise role-appropriate business reporting. Patient-level access is a documented, role-approved exception only when the function and minimum necessary scope are established.

If the purpose or authority is unclear

LAW — must: withhold access or disclosure; do not decide a novel privacy question at the counter. Preserve only needed request facts, then route to privacy/security review. Resume only after documented role/purpose/access approval or another valid legal basis. **GOLDFINCH SAFEGUARD:** retain a concise exception record without PHI beyond what review needs.

Use this operating model

1 | Identify the action	2 | Confirm the authority	3 | Act or stop
Is it entity business, pharmacy/permit, Schedule II, Schedule III–V, receipt, PIC/professional practice, or PHI?	Check local license/permit; local DEA registration; POA/signer and personal CSOS status; assigned workforce role/purpose/minimum necessary; or state ownership approval, as applicable.	If current authority is verified, proceed within scope. If unavailable, expired, revoked, unverified, or disputed: stop, preserve limited facts, route to the listed function, and release only on objective confirmation or approved lawful alternative.

Escalation routes & release conditions

Question / stop	Route	Release only when
Form 222, CSOS, POA, DEA registration, controlled-substance order or receipt	PIC / pharmacy operations, then controlled-substance compliance	Current registrant-linked authority and personal credential are verified, or the responsible function documents a lawful alternative.
Owner/entity/control/PIC/permit status, especially North Dakota	Pharmacy legal / regulatory affairs and state Board review as directed	The applicable state structure and filing/approval status are verified before closing or implementation.
PIC conflict or instruction contrary to practice law	PIC, pharmacy operations, and compliance/legal as needed	A lawful resolution preserves the PIC's professional judgment and accountability.
PHI request, access, or disclosure purpose uncertain	Privacy officer / information security review	Documented role, purpose, minimum-necessary scope and access approval, or other valid disclosure basis.

Authority base

Federal: 21 U.S.C. §822; 21 CFR §§1301.12, 1305.03, 1305.05, 1305.21–.28; HIPAA Privacy/Security Rules, 45 CFR §§164.502, .504(e), .514(d), .308(a)(3), .312. **Minnesota:** Minn. Stat. ch. 151 (including §151.252) and Minn. R. ch. 6800. **North Dakota:** N.D.C.C. ch. 43-15 (including §43-15-35) and N.D. Admin. Code tit. 61. Confirm current Board and DEA instructions for a specific filing or transaction.

Companion tools: State Authority Matrix (action-by-state conditions and sources) and Authority Exception Card (immediate stop/verify/escalate/release).